

Chapter 1

Access, Assessment and Continuity of Care (AAC)

Intent of the chapter

Patients are informed of the services provided by the organisation. Scope of each healthcare services including diagnostic and therapeutic services shall be well defined and the same shall be made available to the patients and their families. Only those patients who can be cared for by the organisation are admitted. Emergency patients receive life-stabilising treatment and are then either admitted (if resources are available) or transferred appropriately to an organisation that has the resources to take care of such patients. Out-patients who do not match the organisation's resources are similarly referred to organisations that have the required resources.

Patients that match the organisation's resources are admitted using a defined process. Patients cared for by the organisation undergo an established initial assessment and periodic re-assessments.

These assessments result in the formulation of a care plan.

The organisation provides laboratory and imaging services commensurate to its scope of services. The laboratory and imaging services are provided by competent staff in a safe environment for both patients and staff. Patient care is continuous and multi-disciplinary. Preventive and promotive healthcare services are part of patient care. Transfer and discharge protocols are well defined, with adequate information provided to the patient. Continuity of patient care is extended to the community through home health care services.

SUMMARY OF STANDARDS

AAC.1.	The organisation defines and displays the healthcare services that it provides.
AAC.2.	The organisation has a well-defined registration and admission process.
AAC.3.	There is an appropriate mechanism for transfer (in and out) or referral of patients.
AAC.4.	Patients cared for by the organisation undergo an established initial assessment.
AAC.5.	Patients cared for by the organisation undergo a regular re-assessment.
AAC.6.	Laboratory services are provided as per the scope of services of the organisation.
AAC.7.	There is an established laboratory quality assurance and safety programme.
AAC.8.	Imaging services are provided as per the scope of services of the organisation.
AAC.9.	There is an established quality assurance and safety programme for imaging services.
AAC.10.	Patient care is continuous and multi-disciplinary.
AAC.11.	The preventive and promotive health services are provided in a safe, collaborative and consistent manner.
AAC.12.	The organisation has an established discharge process.
AAC.13.	The organisation defines the content of the discharge summary.

*This implies that the objective element requires documentation

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c	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Achievement	Commitment	Commitment	Commitment	Commitment
d	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Excellence	Commitment	Excellence	CORE	Commitment	Commitment	Commitment
e		Achievement		CORE	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Commitment	Achievement	Commitment
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Standards and Objective Elements

Standard

AAC.1.

The organisation defines and displays the healthcare services that it provides.

Objective Elements

- Commitment a. The healthcare services being provided are defined and are in consonance with the needs of the community.**

Interpretation: The services provided are defined by senior management and are in consonance with the requirements of the community. The needs of the community shall be considered when planning new services. The needs of the community could be captured through various feedback mechanisms. Changing disease pattern in community is also taken in to consideration for planning new services. (For example, COVID-19 pandemic) However, this does not preclude the organisation from starting new services based on its judgment.

- Commitment b. Each defined clinical service shall have diagnostic and treatment services with suitably qualified personnel who provide out-patient, in-patient, daycare and emergency cover.**

Interpretation: The organisation shall ensure that each defined clinical service i.e. broad speciality and super speciality has suitably qualified medical, nursing and paramedical staff to take care of patient's clinical needs. The said service shall have outpatient services and in-patient services and the consultant(s) shall provide emergency cover.

Infrastructure for diagnostics and treatment facilities shall be guided by regulatory requirements/ guidance from professional bodies where available.

Commitment c. Scope of the clinical services of each department is defined. *

Interpretation: Each department's (super speciality, broad speciality and diagnostic services) scope is defined. The scope could be by inclusion or exclusion in relation to the services practised in the department. The organisation could have a brochure detailing the scope of each department. For example, nephrology department could do all activities like a biopsy, shunts, fistulas, dialysis (haemodialysis and CAPD), etc.

All clinical and diagnostic outsourced services shall be documented and information of the same shall be known to all the staff. This information could also be made available for patients through website, display, brochures etc.

Commitment d. The organisation's defined clinical services are prominently displayed.

Interpretation: Display shall state the names of clinical and diagnostic departments of the organisation. The clinical services so defined shall be displayed prominently in an area visible to all patients and visitors. The display could be in the form of boards, citizen's charter, etc. They shall be permanent. Electronic displays could be used by the organisation. The display shall be at least bi-lingual (State language/language spoken by the majority of people in that area and English). Dissemination of information can be supplemented through the use of brochures, standees and website.

Standard

AAC.2.
The organisation has a well-defined registration and admission process.

Objective Elements

Commitment a. The organisation uses written guidance for registering and admitting patients. *

Interpretation: The Organisation shall prepare a document(s) detailing the mechanism for registration and admission of patients, which shall also include unidentified patients. All patients who are assessed in the hospital shall be registered. Any regulation from the government shall be adhered. The process of registration and admission shall be so designed to avoid duplication in documentation, for example, information once generated in the system shall be available to all the departments within HCO. The organisation could consider mechanisms to verify the identity of the patient during registration. All admissions must be authorised by a doctor. Additional documentation as required shall be included for foreign nationals.

The written guidance addresses out-patients, day-care, in-patients and emergency patients.

The patients and/or family are informed of the salient steps for registration/admission. This could be done through appropriate displays/information on the website.

General consent for treatment is obtained when the patient enters the organisation and its scope is explained. The organisation shall define as to what is the scope of the general consent and the same shall be communicated to the patient and/or his family members. This cannot include consent for invasive procedures or other procedures for which a specific consent is required as per this standard.

CORE

b. A unique identification number is generated at the end of the registration.

Interpretation: The organisation shall ensure that every patient gets a unique number which is generated at the end of registration of the first interaction that the patient has with the organisation. This number shall be used for identification of the patient across the organisation and to ensure continuity of care across the organisation. All hospital records of the patient shall have this number.

“Unique” implies that this is an one-time affair.

Please note that a patient can have only one unique number. However, in the case of multiple visits (OP/IP), a different number could be generated in addition to the above-mentioned unique number. These numbers shall be linked to the unique number to ensure continuity of care.

Commitment c. Patients are accepted only if the organisation can provide the required service.

Interpretation: The staff handling registration and admission need to be aware of the services that the organisation can provide. It is also advisable to have a system wherein the staff is aware as to whom to contact if they need any clarification on the services provided. In case of emergency, life-saving treatment shall be initiated before any decision is taken regarding acceptance.

Commitment d. The written guidance also addresses managing patients during non-availability of beds. *

Interpretation: The organisation is aware of the availability of alternate organisations where the patients may be directed in case of non-availability of

beds. In case the organisation admits these patients in a temporary holding area, it shall ensure that there is adequate infrastructure to take care of these patients. Further, the organisation shall define as to how long patients are kept on temporary beds before a decision to transfer out is taken. The guidance also addresses managing patients when the bed is not available in the desired bed category or unit, and the financial implications explained to the patient of the same.

Achievement e. Access to the healthcare services in the organisation is prioritised according to the clinical needs of the patient.*

Interpretation: Patients with a clinical problem which warrant an earlier response are identified and prioritised in care settings (outpatient, and diagnostic services). For example, a patient waiting in the OPD who complains of giddiness is seen as soon as possible; a vulnerable patient coming for a diagnostic test is fast-tracked. All the staff handling these activities shall be oriented to the applicable guidelines. It is preferable that the organisation develop a mechanism which would help all concerned in identifying such patients. One such mechanism that the organisation could consider is having a visual aid in the form of a sticker placed on the file or on the patient's clothing to help identify such patients.

Standard

AAC.3

There is an appropriate mechanism for transfer (in and out) or referral of patients.

Objective Elements

Commitment a. Transfer-in of patients to the organisation is done appropriately.*

Interpretation: This shall address both planned and unplanned transfers. For unplanned transfers and in case of suspected unstable patients, the organisation could send a suitably trained person with the ambulance. However, this shall be guided by the information received by the organisation. It is a good practice to provide feedback to the referral organisation/doctor on patient's clinical status for transfer in cases.

Commitment b. Transfer-out / referral of patients to another facility is done appropriately.*

Interpretation: Patients needing transfer include those who have come to the emergency but need to be transferred to another organisation or those already admitted but who now require care in another organisation. It also includes patients being shifted for diagnostic tests. The transfer shall be done in consultation with the patient and/or family in a safe manner which includes pre-transfer stabilisation where appropriate, choosing the mode/vehicle for transport, equipment and monitoring required during the transfer. In case the organisation is unable to meet some of the stated requirements, the reasons for the same shall be documented.

Commitment c. During transfer or referral, accompanying staff are appropriate to the clinical condition of the patient.

Interpretation: The staff accompanying shall at least be a trained trauma technologist/emergency technologist/nurse. The staff shall have undergone training in basic or advanced cardiopulmonary resuscitation as appropriate. Further, the staff identified shall be aware of the transfer procedure. A doctor shall accompany an unstable admitted patient who is being transferred out or being shifted for diagnostic purposes.

Commitment d. The organisation gives a summary of the patient's condition and the treatment given.

Interpretation: The organisation gives a transfer summary mentioning the significant findings and treatment given to all patients who are being transferred from the emergency ward/or patients being transferred for diagnostic and therapeutic purposes. In case of a patient being discharged from the organisation and transferred out, a discharge summary is given to such patients, including those patients going against medical advice. A copy of the same shall be retained by the organisation.

Standard

AAC.4.

Patients cared for by the organisation undergo an established initial assessment.

Objective Elements

CORE

- a. **The initial assessment of the out-patients, daycare, in-patients and emergency patients is done in a standardised manner. ***

Interpretation: The organisation shall have a standardised format for initial assessment of patients in the OPD, day-care, in-patient and emergency area. The format shall be designed to ensure that the laid-down parameters are captured.

The initial assessment could be standardised across the hospital, or it could be modified depending on the need of the department. However, it shall be the same in that particular area/unit.

In the emergency, this shall include recording the vital parameters.

For in-patients, this shall cover history, examination, including vital signs and documentation of any drug allergies. It shall mention the provisional diagnosis. If a detailed assessment has been done earlier (either in OPD or emergency on the same day), it need not be written in detail again. However, there shall be a comment linking the assessment to the earlier assessment, and the findings of all such assessments shall be reviewed and/or verified.

Initial assessment shall include reconciliation of medications for IP patients.

Abridged documentation may be used for daycare as appropriate. This includes patients coming for dialysis.

Refer to COP.10.d. and COP.11.e.

- Commitment b. The initial assessment is performed by qualified personnel. ***

Interpretation: The organisation determines who can do what assessment. Various categories of staff could do the assessment. Caregivers perform initial assessment within their scope of practice, registration and applicable laws and regulations. Psychological, spiritual, cultural, social and economic aspects of initial assessment for in- patients could be done by various healthcare professionals.

The organisation defines the process by which contents of the assessment (initial assessment and diagnostic) performed by a qualified and privileged healthcare professional associated with the hospital is accepted in to the system of patient care. For example, patients referred by visiting consultant.

Commitment c. The initial assessment is performed within a time frame based on the needs of the patient. *

Interpretation: The organisation shall define and document the time frame within which the initial assessment is to be completed concerning day-care, in-patients and emergency; and the same shall be implemented. In the case of in-patients, the time frame shall be from the time the patient arrives at the ward until the initial assessment is completed. The maximum time within which the initial assessment is completed for an in-patient is 24 hours. Patients may be assessed earlier depending upon the clinical need.

In the case of an emergency, the time frame shall be from the time the patient arrives at the emergency until the initial assessment is completed.

Commitment d. Initial assessment of daycare and in-patients includes nursing assessment, which is done at the time of admission and documented.

Interpretation: The initial assessment shall identify the nursing needs and also help identify any special needs of the patient. It shall be completed within a defined time frame. A checklist or template could be used for the same. Abridged nursing documentation may be used for daycare as appropriate. It could be specific to the concerned speciality/type of admission.

CORE e. The initial assessment for in-patients results in a documented care plan.

Interpretation: The care plan shall be documented by the treating doctor or by a doctor member of the treating team in the patient record and followed. The care plan is prepared and documented based on initial assessment and result of diagnostic tests if available. It shall include a provisional diagnosis/differential diagnosis, relevant diagnostic investigations when required; initial treatment suggested and specific instructions if any. The care plan shall be subject to modifications or changes at reassessments. The care plan reflects the desired results of the treatment, care or service.

Achievement f. The care plan is countersigned by the clinician in-charge of the patient within 24 hours.

Interpretation: The treatment of the patient could be initiated by a junior doctor but the same shall be countersigned and authorised by the treating doctor within 24 hours. The clinician in charge implies the treating doctor.

Excellence

- g. The care plan includes the identification of special needs regarding care following discharge.**

Interpretation: Identification of special needs is critical in certain groups of patients like extremes of age, restricted mobility, continuing nursing and rehabilitation needs, assistance with activities of daily living etc. The organisation could begin the process of identifying special discharge needs early in the assessment process.

Standard**AAC.5.**

Patients cared for by the organisation undergo a regular re-assessment.

Objective Elements**CORE**

- a. Patients are re-assessed at appropriate intervals to determine their response to treatment and to plan further treatment or discharge.**

Interpretation: After the initial assessment, the patient is reassessed periodically, and this is documented in the case sheet. Re- assessment shall be done for all care givers as applicable. The frequency may be different for different areas based on the setting, and the patient's condition, for example patients in ICU need to be reassessed more frequently compared to a patient in the ward. Reassessments shall also be done in response to significant changes in patient's condition. Every patient shall be reassessed at least once every day by the treating doctor or a doctor from the treating team. Reassessments shall also be done for day-care patients (before discharging) or patients awaiting admission/bed.

Caregivers perform reassessment within their scope of practice, registration and applicable laws and regulations.

Commitment

- b. Out-patients are informed of their next follow-up, where appropriate**

Interpretation: The information could be either in terms of a specific date or after a certain period (weeks/months) and shall be documented in the medical record/OP consultation sheet. This may not be applicable in cases where the patient has come for just an opinion or the patient's condition does not warrant a repeat visit.

- Commitment c. For in-patients during re-assessment, the care plan is monitored and modified, where found necessary.**

Interpretation: The care plan is monitored to assess its effectiveness in achieving the desired results of the treatment, care or service. The care plan shall be dynamic and modified where necessary according to the patient's condition by the treating doctor or a doctor member of the treating team. The change in the care plan is documented in the medical record. Different sections of the medical record, such as progress notes, doctor's orders or medication charts, could show evidence of the change in the care plan.

- Commitment d. Staff involved in direct clinical care document re-assessments.**

Interpretation: Actions taken under reassessment are documented. The staff could be the treating doctor or any member of the team as per their domain of responsibility of care. At a minimum, the documentation shall include vitals, systemic examination findings and medication orders. The nursing staff can document a patient's vitals. Only phrases like "patient well"; "condition better" would not be acceptable.

- Commitment e. The organisation lays down guidelines and implements processes to identify early warning signs of change or deterioration in clinical conditions for initiating prompt intervention. ***

Interpretation: The staff uses defined physiological parameters to identify clinical deterioration. These may include assessment of vital parameters, airway, circulation, neurological status, and any other concerns felt by the staff or patient/patient's family. The parameters may be tailored to suit the needs of the speciality and the age-group. The organisation has a mechanism whereby this information is made available to appropriate medical personnel to initiate prompt and appropriate actions. The effectiveness of early warning system shall be monitored.

To be correlated with COP.5., COP.6., HRM.1. and HRM.5.

Standard

AAC.6.

Laboratory services are provided as per the scope of services of the organisation.

Objective Elements

- Commitment a. Scope of the laboratory services is commensurate to the services provided by the organisation.**

Interpretation: The organisation shall ensure availability of laboratory services, commensurate with the healthcare services it offers. For example, a cardiac care organisation must necessarily have facilities for cardiac enzymes. The organisation shall ensure that these services are available round the clock, and patient care is not disrupted. Test results required for emergency management must be available within its premises.

- Commitment b. The infrastructure (physical and equipment) is adequate to provide the defined scope of services.**

Interpretation: Laboratory shall have adequate space and equipment to meet its defined scope of services. Reports shall not get delayed due to lack of adequate equipment.

The layout of the laboratory prevents cross-contamination.

- Commitment c. Human resource is adequate to provide the defined scope of services.**

Interpretation: The number of laboratory personnel shall be commensurate with the workload, with sufficient staff for each shift and emergencies. Reports shall not get delayed due to lack of adequate human resource (including personnel authorised to report results).

- Commitment d. Qualified and trained personnel perform and supervise the investigations and report the results.**

Interpretation: The staff employed in the laboratory shall be suitably qualified (appropriate degree) and trained to carry out the tests. Pathologist, Microbiologist and Biochemist supervise the staff. Statutory requirements regarding authorised signatory shall be adhered.

- Commitment e. Requisition for tests, collection, identification, handling, safe transportation, processing and disposal of a specimen is performed according to written guidance. ***

Interpretation: The organisation has documented guidance for requisition, collection, identification, handling, safe transportation, processing, and disposal of the specimen, to ensure the safety of the specimen till the tests and retests (if required) are completed (observing standard and special precautions). The organisation shall ensure that the unique identification number is used for identification of the patient. Also, it could use another number (for example, lab number) to identify the sample. The disposal of waste shall be as per the statutory requirements (Bio-medical waste management and handling rules).

- Commitment f. Laboratory results are available within a defined time frame. ***

Interpretation: The organisation shall define the turnaround time for all tests. The organisation shall ensure the availability of adequate staff, materials and equipment to make the laboratory results available within the defined time frame. The turnaround time could be different for different tests and could be decided based on the nature of the test, the criticality of test and urgency of test result (as desired by the treating doctor).

Refer to AAC.6.b., AAC.6.c. and AAC.6.d.

- Commitment g. Critical results are intimated to the person concerned at the earliest. ***

Interpretation: The laboratory shall establish its biological reference intervals for different tests. The laboratory shall establish and document critical limits for tests which require immediate attention for patient management, and the same shall be documented. If it is not practical to establish the biological reference interval for a particular analysis, the laboratory shall carefully evaluate the published data for its reference intervals. Critical results of outsourced investigations are also included.

Laboratory in-charge identifies suitable personnel to report critical results. The critical test results shall be communicated to the person concerned from the treating team (treating doctor/doctor member of treating team/ward nurse) at the earliest, but not later than one hour of completion of test/report being ready.

The intimation includes the name of the patient; Unique ID; date and time of sample collection; test name, result, measure unit and reference range; operator identity of who has communicated the value; the identity of the recipient; read-back and date and time of acknowledgement. The intimation shall be documented.

In the case of electronic health systems, system-generated critical result reporting can supplement the physical reporting of critical results.

Commitment h. Results are reported in a standardised manner.

Interpretation: At a minimum, the report shall include the name of the organisation (or in case of outsourced laboratory, the name of the same), the patient's name, the unique identification number, reference range of the test (where applicable) and the name and signature of the person reporting the test result. All reports from the outsourced laboratory shall incorporate these features, and the organisation shall not alter/modify anything in the report. In the case of outsourced test results, the same shall be either on the outsourced laboratory's letterhead or on the organisation's letterhead. If it is done on the organisation's letterhead, it shall include at least the name of the outsourced laboratory, date and reference number of the report given by the outsourced laboratory.

Achievement i. There is a mechanism to address the recall / amendment of reports whenever applicable.*

Interpretation: These could include recall for errors due to pre-analytical, analytical and post-analytical factors. Whenever there is a recall of a particular report, it is withdrawn from clinical areas, medical records, Laboratory Information System (LIS) and Hospital Information System (HIS). If already issued to the patient, the amended report is made available to the patient with the caution to ignore the earlier one. The same shall be documented. Placement of corrected report in all these areas is also evidenced. Corrective and preventive action is implemented as appropriate, based on detailed analysis.

Commitment j. Laboratory tests not available in the organisation are outsourced to the organisation(s) based on their quality assurance system. *

Interpretation: The organisation has written guidance for outsourcing tests for which it has no facilities. This shall include:

- A list of tests for outsourcing.
- Identity of personnel in the outsourced facilities to ensure safe and timely transportation of specimens and completing of tests as per requirements of the patient concerned and receipt of results at the organisation.
- Manner of the packaging of the specimens and their labelling for identification and this package shall contain the test requisition with all details as required for testing.
- A methodology to check the performance of service rendered by the outsourced laboratory, as per the requirements of the organisation.

The organisation shall have a Memorandum of Understanding (MoU)/agreement for the same, which incorporates quality assurance and requirements of this standard.

Refer to ROM.6.e.

Standard

AAC.7.

There is an established laboratory quality assurance and safety programme.

Objective Elements

Commitment a. The laboratory quality assurance programme is implemented. *

Interpretation: The organisation has a documented quality assurance programme to ensure accuracy and precision. Programme shall also include point of care testing (POCT). The written guidance covers pre-analytic, analytic and post-analytic phases of the testing cycle. The programme includes test standardisation internal quality control and external quality assurance, or inter laboratory testing (as applicable). There is a mechanism to obtain feedback from various stakeholders to evaluate the laboratory services at least once a year.

The programme addresses verification and/or validation of test methods.

The programme includes periodic calibration and maintenance of equipment.

The programme includes the documentation of corrective and preventive actions whenever deviations are observed.

A good reference is ISO 15189:2022 and NABL 112.

Commitment b. The programme ensures the quality of test results through internal quality control.*

Interpretation: Ensuring the quality of test results includes performing internal quality controls to ensure precision and repeatability for all test parameters (quantitative and qualitative) and peer review, where relevant. The programme includes comparability of results when more than one measuring systems are used.

A good reference guide is NABL 112

Corrective and preventive actions are taken to address the deviations.

Commitment c. Laboratory participates in proficiency testing / external quality assurance scheme.

Interpretation: The laboratory shall participate in external quality assessment (EQA)/ Proficiency testing (PT). Based on the EQA/PT evaluation report the laboratory shall implement and document Corrective actions for the outliers. For Some test EQA/PT is not a physical option for one or more of the following reasons:

- Non availability of a formal national PT programme for analytes of interest;
- Only few laboratories performing the test;
- The analyte to be measured is unstable for example blood gases, ammonia, G6PD;
- Control material of the same matrix is not available;

For those tests where formal EQA is not available, the laboratory shall adopt alternate approaches to validate performance.

This could be replicate testing examination of split samples within the laboratory, use of reference methods and material where available and exchange of sample with other accredited laboratories for inter laboratory comparisons.

A good reference guide is NABL 112

Excellence d. The programme addresses the clinico-pathological meeting(s).

Interpretation: The organisation conducts the clinicopathological meeting(s) at pre-defined intervals for correlating the histopathology reports with referring clinicians and uses the same as a tool for improving quality.

Commitment e. The laboratory safety programme is implemented. *

Interpretation: Laboratory safety manual is available in the laboratory. This takes care of the safety of the workforce as well as the equipment available in the laboratory. It shall be in consonance with the risks and hazards identified. The manual shall incorporate the appropriate Material Safety and Data Sheet (MSDS). Safety Programme includes, safe handling of equipment and accessories including their disinfection. The laboratory safety programme could be as per Occupational Health and Safety Management System.

This programme is aligned with the organisation's safety programme.

Commitment f. Laboratory personnel are appropriately trained in safe practices.

Interpretation: All the laboratory staff shall undergo training regarding safe practices in the laboratory as well as in the relevant MSDS. The training need identification must be made commensurate with the job description of the staff.

Commitment g. Laboratory personnel are provided with appropriate safety measures.

Interpretation: Adequate safety measures are available in the lab, for example PPE, dressing materials, disinfectants, fire extinguishers etc. It shall address safety issues at all levels. All laboratory personnel shall always adhere to standard precautions. All lab staff shall be appropriately immunised.

Standard

AAC.8.

Imaging services are provided as per the scope of services of the organisation.

Objective Elements

CORE

a. Imaging services comply with legal and other requirements.

Interpretation: The organisation is aware of the legal and other requirements of imaging services and the same are documented for information and compliance by all concerned in the organisation. The organisation maintains and updates its compliance status of legal and other requirements in a regular manner. All the statutory requirements are met with, such as Atomic Energy Regulatory Board (AERB) clearance, dosimeters, lead shields, lead aprons, signage, display as per Pre-conception and Pre-natal diagnostics techniques (PC-PNDT) Act, reports to the competent authority, etc. The organisation shall have a Radiation Safety Officer (of appropriate level).

Commitment b. Scope of the imaging services is commensurate to the services provided by the organisation.

Interpretation: The organisation shall ensure availability of imaging services commensurate with the healthcare services it offers. For example, an organisation providing neurosurgery services, including head injuries, shall have facilities for CT (Computed Tomography) scan.

Imaging services may be provided within the organisation, outsourced to another organisation or both. The key aspect that needs to be ensured is the safe transfer of the patient and the imaging reports being available on time.

The organisation shall ensure that these services are available round the clock, and patient care is not disrupted.

Imaging modalities required for emergency management could preferably be available within its premises.

Commitment c. The infrastructure (physical and equipment) and human resources are adequate to provide for its defined scope of services.

Interpretation: Imaging services shall have adequate space and equipment to meet their defined scope of services. Reports shall not get delayed due to lack of adequate equipment or human resources (including personnel authorised to report results).

Commitment d. Qualified and trained personnel perform, supervise and interpret the investigations.

Interpretation: AERB guidelines could be used as a reference document for radiation-based imaging.

Commitment e. Imaging results are available within a defined time frame. *

Interpretation: The organisation shall document turnaround time of imaging results for all modalities. The organisation shall monitor the waiting times, time taken to perform the tests and time taken to prepare the reports of the tests for all modalities; for in-patient, outpatient and emergency. The defined timeframes could be different for different type of tests and could be decided based on the nature of the test, modality, criticality of the test and the urgency of the test result (as required by the treating doctor).

Commitment f. Critical results are intimated immediately to the personnel concerned. *

Interpretation: The organisation shall define and document the critical results which require immediate attention for patient management and the same shall be documented, for example ectopic pregnancy. The critical results must be

documented for each modality of imaging. Critical results of outsourced investigations shall also be intimated.

The critical test results shall be communicated to the person from the treating team (treating doctor/doctor member of treating team/ward nurse) at the earliest, but not later than one hour after completion of the test/report being ready. Imaging services in-charge identifies suitable personnel to report critical results.

The intimation includes the name of the patient; Unique ID; date and time of imaging; investigation name, result; operator identity of who has communicated the value; the identity of the recipient; read-back and date and time of acknowledgement. This shall be documented.

In the case of electronic health systems, system-generated critical result reporting can supplement the physical reporting of critical results.

Commitment g. Results are reported in a standardised manner.

Interpretation: At a minimum, the report shall include the name of the organisation (or in case of an outsourced imaging centre, the name of the same), the patient's name, the unique identification number, and the name and signature of the person reporting the test result. In case of teleradiology, there shall be the name of the reporting doctor and a remark to that effect. It shall also include the name of the reporting organisation if outsourced to an organisation. All reports from the outsourced imaging centre shall incorporate these features, and the hospital shall not alter/modify anything in the report. The report shall be in the prevailing context, taking into account the clinical details and results of any previous imaging.

Achievement h. There is a mechanism to address the recall / amendment of reports whenever applicable. *

Interpretation: These could include recall for errors at all levels. Whenever there is a recall of a particular report, withdrawal from clinical areas, medical records, RIS and HIS shall be ensured. If already issued to the patient, the amended report is made available to the patient with the caution to ignore the earlier one. The same shall be documented. Placement of corrected report in all these areas is also evidenced. Corrective and preventive action is implemented as appropriate based on detailed analysis.

Commitment i. Imaging tests not available in the organisation are outsourced to the organisation(s) based on their quality assurance system.

Interpretation: The organisation has written guidance for outsourcing tests for which it has no facilities. This shall include:

- List of tests for outsourcing,
- Identity of personnel in the outsourced facilities to ensure the safe transportation of patients and completing of imaging results,
- Manner of identification of patients and the test requisition with all details as required for testing and a methodology to check the selection and performance of service rendered by the outsourced imaging facility as per the requirements of the organisation.
- Reporting of critical results.
- Turnaround time for emergency requests.
- Turnaround time for routine requests.
- Prioritisation for conduct of investigations in case of urgent requests.

The organisation shall have an MoU/agreement for the same, which incorporates quality assurance and requirements of this standard.

Refer to AAC.3.c. and ROM.6.e.

Standard

AAC.9.

There is an established quality assurance and safety programme for imaging services.

Objective Elements

Commitment a. The quality assurance programme for imaging services is implemented. *

Interpretation: The quality assurance programme shall be a comprehensive programme addressing equipment, protocols, surveillance and safety. Also, statutory (AERB) requirements will have to be met.

Quality assurance programme includes tests for imaging equipment.

Some examples of tests for imaging equipment include congruence of optical and radiation field, focal spot size, output consistency, leakage rate, magnetic field homogeneity, slice position accuracy, Phantom checks etc. The tests, as applicable, shall be performed.

Quality assurance program for nuclear medicine (PET scan / Gamma camera / SPECT CT) and radiotherapy (Linear accelerator, Brachy Therapy) shall be implemented as per AERB guidelines.

Safety equipment shall undergo testing, as stated in AAC.9.i.

Quality assurance programme includes the review of imaging protocols.

The review of imaging protocols ensures optimum image quality with minimum possible dosage to the patient(s). The imaging protocols shall be in accordance with guidelines given by professional bodies/academic literature and where relevant based on the clinical diagnosis.

Achievement b. A system is in place to ensure the appropriateness of the investigations and procedures for the clinical indication.

Interpretation: The investigation orders are screened before performing the imaging or interventional procedure to ensure that they are an appropriate investigation (as per current best practice guidelines and patient safety) for the clinical indication. Otherwise, alternate investigations are offered in consultation with the treating doctor. For example, if breast abscess is suspected as the cause of fever and breast lump in a lactating 25 years old lady, mammography is inappropriate, as it would not reveal the breast abscess; ultrasound scan of the breast will be the appropriate investigation.

The organisation could maintain a record to capture instances wherein the requested imaging test was modified after consultation. It could perform an internal audit to capture the effectiveness of the system and discuss the same in clinic-radiological meetings.

Achievement c. The programme addresses periodic internal / external peer review of imaging results using appropriate sampling

Interpretation: A peer review system is in place to review the imaging results of CT and MRI. The peer review shall be done in a structured manner and the sample size and periodicity for each modality shall be defined by the organisation. However, at a minimum, this shall be one percent. The peer review can be performed by the head of the department or by a group of peers, with blinding of the original reports. Discrepancies in the reports will be graded on the severity and impact on changes in patient management strategy, and the corrective and preventive actions taken to minimise these will be documented. One such scoring scale is RADPEER. The purpose is to prevent errors in future, and continuous

quality improvement rather than computation or error rates of the individuals. The results of such reviews could be discussed with all stakeholders in “discrepancy meetings”, and the same shall be documented.

Excellence d. The programme addresses the clinico-radiological meeting(s).

Interpretation: The organisation conducts the clinico-radiological meeting(s) at pre-defined intervals for correlating the imaging results(at a minimum CT and MRI) with referring clinicians and uses the same as a tool for improving the quality of its imaging results.

Commitment e. The programme includes the documentation of corrective and preventive actions. *

Interpretation: In case of any deviations noted from the laid down quality assurance programme, the organisation shall institute corrective and preventive actions as may be appropriate.

Commitment f. The radiation-safety programme is implemented.*

Interpretation: The programme shall be in consonance with the guidelines laid down by AERB. The programme also includes the implementation of "As Low As Reasonably Achievable" (ALARA) principle in investigations involving radiation and screening of those patients who are at high risk for radiation. For example, routine daily chest radiograph for all ICU patients cannot be justified, more so in paediatric or neonatal ICUs. Further, CT scan protocols are modified to use the lowest exposure parameters to maintain the image quality appropriate for the clinical indication. For example, CT for ureteric calculi can be done with low dose while renal tumour will require a higher dose.

Appropriate safety precaution as per AERB guidelines with respect to nuclear medicine (PET scan / Gamma camera / SPECT CT) and radiation oncology (Linear accelerator, Brachy Therapy) shall be followed .

This programme is aligned with the organisation's safety programme

Commitment g. Patients are appropriately screened for safety / risk before imaging.

Interpretation: Patients in the child-bearing age group who need to be exposed to radiation shall be screened for pregnancy. Patients undergoing MRI shall be screened for any magnetic substance. Screening shall be documented. The screening shall also apply to attendants accompanying the patient/child into the imaging area.

Informed consent shall be taken for contrast injection, moderate-deep sedation, interventional procedures, and whenever the higher risk due to imaging is found on risk screening.

Refer to PRE.4.a.

Commitment h. Imaging personnel and patients use appropriate radiation- safety and monitoring devices where applicable.

Interpretation: Shielding of body parts of staff and patients, attendants shall be adhered to using appropriate aprons and shields. The number of such devices shall be adequate to ensure that all workers have proper protection. Staff directly working with radiation sources shall possess and use Thermo Luminescent Dosimeters (TLD) badges.

Commitment i. Radiation-safety and monitoring devices are periodically tested, and results are documented. *

Interpretation: Protective devices, for example lead aprons, shall be exposed to X-ray or fluoroscopy or CT scout view for verification of cracks and damages. The monitoring is done periodically as per national guidelines. The images of the same shall be stored (either physical or electronic). Where appropriate corrective and/or preventive action shall be taken and documented.

TLD badges must be replaced by the fresh badges supplied by accredited laboratories according to the frequency recommended by AERB.

Commitment j. Imaging and ancillary personnel are trained in imaging safety practices and radiation-safety measures.

Interpretation: Imaging safety practices include training of imaging and ancillary personnel on MRI safety, kinking of tubes, fall prevention, and handling patients in

the imaging areas. Radiation safety measures refer to the steps taken to protect the patient and staff from unwanted radiation.

The ancillary staff refers to those staff who are posted in the imaging service who support the radiologist, radiographers, MRI/CT technologists in the activities in the imaging service. These staff may include nurses, helper staff, stretcher-bearers, housekeeping, security, etc.

Commitment k. Imaging signage is prominently displayed in all appropriate locations.

Interpretation: This includes safety signage and display of signage as required by regulatory authorities. This includes procedure rooms (for example ERCP) and operation theatres.

Standard

AAC.10.

Patient care is continuous and multi-disciplinary.

Objective Elements

Commitment a. During all phases of care, there is a qualified individual identified as responsible for the patient's care.

Interpretation: For all patients cared for by the organisation, there is a qualified doctor identified as responsible for care. Although a team may provide care, the hospital record shall identify a doctor as being responsible for patient care.

Commitment b. Patient care is coordinated in all care settings within the organisation.

Interpretation: Patient care is coordinated among various care-providers in a given setting viz OPD, Emergency, IP, ICU, etc. The organisation shall ensure that there is effective communication of patient requirements amongst the care-providers in all settings.

Commitment c. Information about the patient's care and response to treatment is shared among medical, nursing and other care providers.

Interpretation: Information sharing could be done through entries either in the case-sheet or in electronic medical records (EMR). The patient's record(s) is available to the authorised care-providers to facilitate the exchange of information. Other modalities for sharing of information are team meetings, grand rounds etc.

Refer to COP.1.e.

CORE d. The organisation implements standardised hand-over communication during each staffing shift, between shifts and during transfers between units / departments.

Interpretation: Change of shift and change of unit handover by doctors, and nurses involved in direct patient care shall be standardised and documented. Information shared shall consist of the patient's current condition, recent changes in condition, ongoing treatment and possible changes or complications. Situational briefing techniques such as the SBAR (Situation, Background, Assessment and Recommendation) process can provide a standard framework for handovers. Read-back, multidisciplinary rounds, teach-back, involving patients and families in the process of care are other modalities to ensure and streamline handover communication.

Commitment e. Patient transfer within the organisation is done safely.

Interpretation: The organisation shall ensure that intra-organisation transfers are done adhering to safe practices. The patients shall be transported safely, and a proper handover and takeover shall be documented. Patients are transported in a safe and timely manner to and from the imaging services. The system shall be in place to ensure that the conduct of imaging tests is not delayed due to delay in transportation. This shall also address the transfer of unstable patients.

Commitment f. Referral of patients to other departments / specialities follow written guidance.*

Interpretation: Referral could be for opinion, co-management or takeover. The referral note shall mention the reason for the referral. It could be graded into

immediate, urgent, priority or routine category. All referrals shall be based on clinical significance and for a better outcome. All referrals shall be seen within a defined timeframe. The timeframe could be different based on the urgency of referral. The organisation has written guidance for the referral of patients to other departments or specialities.

- Achievement g. The organisation ensures predictable service delivery by adhering to defined timelines and informs the patient / family and / or caregiver whenever there is a change in schedule.**

Interpretation: The organisation has defined timelines (for example Laboratory, Radiology, OPD waiting time) to ensure predictable service delivery. Patients are informed of the same. Whenever there is a deviation from the defined timelines, the patient/family are informed of the change in schedule. The organisation shall also inform the caregiver to ensure that the effectiveness of care is not compromised.

- Excellence h. The organisation has a mechanism in place to monitor whether an adequate clinical intervention has taken place in response to a critical value alert.**

Interpretation: The attending clinician shall respond to a critical value alert. Evidence of the same shall be documented in the progress notes/medication orders. The organisation has a mechanism to periodically review the intervention to assess for timeliness and appropriateness of the response. In the case of outpatient, efforts will be taken to alert the patient or family about the critical values.

Standard

AAC.11.

The preventive and promotive health services are provided in a safe, collaborative and consistent manner.

Objective Elements

- Commitment a. Written guidance governs the implementation of preventive and promotive care as per the scope of services.***

Interpretation: Preventive care shall focus on measures taken to prevent the onset of diseases or health conditions before they occur for example immunization, health screening, health education, and lifestyle counselling, based on age, gender and medical history. Promotive care shall involve efforts to promote overall health and well-being among patients, families and community for example, promoting healthy diet, regular exercise, stress management, mental health support and initiatives to improve overall quality of life through patient and family education, health literacy and community activities.

- Commitment b. Organisation shall define evidenced based and contextual age-appropriate screening for non-communicable diseases.**

Interpretation: For prevention of non-communicable diseases evidence-based screening of patients at an appropriate age shall be advised, for example screening for various cancers, diabetes, osteoporosis. The frequency of screening may increase if the predisposing factors are applicable.

- Commitment c. Mental health screening and appropriate intervention is advised for patients wherever applicable.**

Interpretation: Mental health screening could be done by using validated tools, for example Patient health questionnaire (PHQ-9) for depression, distress thermometer for oncology patients, etc.

De-stressing intervention may be considered for patients seeking healthcare, for example Yoga and meditation.

- Commitment d. Evidence based and contextual paediatric and adult immunisation shall be advised wherever applicable.**

Interpretation: Universal Immunisation Program shall be adhered for all neonates and paediatric patients (Catch-up immunization for children). Adult immunisation may focus on covering all preventable diseases like Pneumococcal Pneumonia, HPV, Influenza, COVID, Meningococcal Disease, etc adhering to evidence based guidelines.

- Commitment e. A multi-disciplinary approach is adopted in imparting health education on life-style modifications.**

Interpretation: Multi-disciplinary approach may include advise by clinician, nurse, dietician, physiotherapist, occupational therapist, psychologist, medico social worker etc. This shall include incorporating exercises, training in ADL's and effective pain and inflammation management. Healthcare providers educate and aim to optimize the health and well-being of their patients, particularly in the context of geriatric care. Family members shall also be educated towards cessation of tobacco, alcohol and substance abuse.

Therapeutic exercises involve planned and systematic exercises designed to promote physical strength, flexibility and mobility. They may be tailored to the patient's specific needs and may be utilized for rehabilitation after surgery or certain chronic conditions.

Standard

AAC.12.

The organisation has an established discharge process.

Objective Elements

- Commitment a. The patient's discharge process is planned in consultation with the patient and / or family.**

Interpretation: The patient's treating doctor determines the readiness for discharge during regular reassessments. The same is discussed with the patient and family.

- Commitment b. The discharge process is coordinated among various departments and agencies involved (including medico-legal and absconded cases).***

Interpretation: The discharge procedures are documented to ensure coordination amongst various departments, including accounts so that the discharge papers are completed well within time. For medico-legal cases (MLC), the organisation shall ensure that the police are informed about the discharge.

Commitment c. Written guidance governs the discharge of patients leaving against medical advice.*

Interpretation: The treating doctor shall explain the consequences of this action to the patient/attendant. The written guidance could address the reasons for leaving against medical advice (LAMA) for any possible corrective and/or preventive action by the organisation.

Commitment d. A discharge summary is given to all the patients leaving the organisation including patients leaving against medical advice.

Interpretation: The organisation hands over the discharge summary and reports to the patient/attendant in all case, and a copy is retained in the medical record. In LAMA cases, the patient's right to refuse treatment and his/her request to leave the organisation is respected, the declaration of the patient/attendant is to be recorded in a proper format, and a discharge summary and all reports are handed over as usual. The terminology used to refer to such patients may differ, but the intent of issuing the discharge summary with reports remains the same. In case of death, a death summary is given to the next of kin/relatives.

Achievement e. The organisation adheres to planned discharge.

Interpretation: Discharge is planned at least 24 hours in advance. Planning shall include preparation of the draft discharge summary, refund of medications, patient education on continued care, identified special needs, for example requirement of any special equipment, devices, transportation requirement (wheelchair), safe and effective use of medical equipment. Unplanned discharges are minimised.

Refer to AAC.4.g.

Achievement f. The care shall be provided by expanding access to health practices through domiciliary visits, wherever applicable.

Interpretation: Wherever applicable, trained healthcare workers visit the patients at their residences to deliver medical care, treatment and/ or support. Domiciliary visit is conducted to address the unique needs and challenges faced by patients such as managing chronic conditions, fall prevention, memory care, nutrition, exercise, and other health concerns. For palliative care patients, home visit for change of catheters, and/or any other support care may be conducted.

Commitment g. The organisation monitors the discharge time, sets appropriate benchmarks and makes continual improvement.

Interpretation: The organisation defines the time taken for discharge. The timeframe could be defined based on pay or mix, for example, cash, insurance, corporate. The organisation shall conform to the defined timeframe for discharge. The time taken for discharge is monitored for delays. Reasons for delays are identified and improvement activities performed. The start-point for calculating the time taken for discharge is when the treating doctor declares that the patient is fit for discharge. The endpoint is when the patient vacates the bed.

Standard

AAC.13. The organisation defines the content of the discharge summary.

Objective Elements

Commitment a. A discharge summary is provided to the patients at the time of discharge.

Interpretation: The discharge summary shall be signed by the treating doctor or doctor member of the treating team. Patient/family acknowledges the receipt of the same.

Commitment b. Discharge summary has a standardised content.

Interpretation: Discharge summary shall have following minimum content:

- Patient's name
- Unique identification number
- Name of the treating doctor
- Date of admission and date of discharge
- Reasons for admission
- Significant findings, diagnosis and the patient's condition at the time of discharge
- Information regarding investigation results
- Any procedure performed
- Medication administered

- Any other treatment given.

In addition to the name of the treating doctor, it could also have the name of the other consultants involved in the treatment.

Commitment c. Discharge summary contains follow-up advice, medication and other instructions in an understandable manner.

Interpretation: This shall also incorporate preventive aspects, where appropriate. The organisation ensures that the follow-up advice, medication and other instructions are explained to the patient and or relatives in a language and manner that they understand. Medical terms like BD, TID, QID shall not be used.

Other instructions include safe and effective use of medical equipment at home for example CPAP, Nebulizer, Rehab equipment. For post operative patients summary shall include wound care instructions and pressure ulcers care after discharge.

Refer to AAC.4.g.

Commitment d. Discharge summary incorporates instructions about when and how to obtain urgent care.

Interpretation: The discharge summary shall contain advice on 'when' the patient shall seek urgent care. This information shall be specific to the patient's diagnosis and clinical condition at the time of discharge. For example, development of fever, bleeding/discharge from the operative site. The advice could be in the form of what medicines to take, when to consult a doctor or how to seek medical help and contact number of the hospital/doctor. It is a good practice to take follow up of post operative and at risk patients from patient safety point of view. The organisation ensures that instructions about when and how to obtain urgent care are explained to the patient and/or relatives in a language/manner that they understand.

Refer to PSQ.4.d.

Commitment e. In case of death, the summary of the case also includes the cause of death.

Interpretation: In case the cause of death is not clear, a post-mortem is performed (for example MLC) and the same shall be documented.

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